



BRIEFS FROM AE

# Tri-County Market Intelligence Kootenai · Bonner · Spokane Counties

Prepared for Barry Burris, NMD  
Accelerated Experiences, LLC — July 2026



## Executive Summary

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**868**

Referral contacts,  
12 states

**71**

Local trade-area  
contacts

**17**

Local NMD  
competitors

**56**

Total tri-county  
competitors, all types

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Barry's referral network (868 contacts, spanning 12 states) has been re-audited and re-tagged. The relevant finding for day-to-day strategy isn't the size of the list — it's that only a small slice of it sits inside Barry's actual trade area. Of 92 naturopathic-credentialed contacts on the full list, **17 are true local competitors** (same license, same drive-time, same patient pool) and **75 are extended-network peers** — professional colleagues in other states who will never compete for a Coeur d'Alene patient, but are exactly the kind of people worth a phone call, a conference introduction, or a referral relationship if Barry ever expands, teaches, or refers a relocating patient. 106 entries across the list are flagged “thin” — real businesses with too little public documentation to act on confidently, not fabricated or removed, just marked for a manual check before outreach.

Beyond the referral list, fresh research this cycle mapped the **actual competitive battlefield**: 56 practices across Kootenai, Bonner, and Spokane counties that compete with Barry for the same patient, once you count not just other NMDs but the hormone clinics, med-spas, peptide-therapy shops, and concierge primary-care practices chasing the same “I want to feel better and live longer” customer. That's the real number that matters for positioning — and it's roughly triple the naturopath-only count most people would assume.

The three counties are not one market. Kootenai is the home turf — dense, competitive, growing fast. Bonner is a smaller, older, more seasonal market where a longevity-forward brand still has real room. Spokane is the big one next door — six times Kootenai's population, more institutional competition, and at least one direct competitor (Clinic 5C) that has already expanded into Coeur d'Alene itself.

This brief lays out the competitive landscape county by county and combined, a target-partner profile for referral development, a plain-English breakdown of who Barry's most probable patients actually are, and what to do with all of it.



## Part 1 — Referral Network: Re-Audit Results

**What changed:** every one of the 868 rows on Barry's referral list now carries three new tags in the live Directory:

- **Zone** — *local* (inside Kootenai, Bonner, or Spokane counties) or *extended* (everywhere else in the 12-state network)
- **Competitor** — *true* only for local-zone Naturopathic Doctors who see the same patients Barry does
- **Thin** — flagged where public documentation was too sparse to act on with confidence, so nothing gets outreach without a manual look first

| Metric                                         | Count |
|------------------------------------------------|-------|
| Total referral network                         | 868   |
| Local trade area (Kootenai + Bonner + Spokane) | 71    |
| Extended network (other states/regions)        | 797   |
| Naturopathic Doctors, total                    | 92    |
| — Local competitors                            | 17    |
| — Extended-network peers                       | 75    |
| Flagged "thin" (needs manual verification)     | 106   |

This directly answers the “he has obscure businesses in weird states” problem: those obscure out-of-state entries aren't wrong, they're just correctly filed as extended network now — visible, filterable, and clearly separated from the 71 that actually matter for day-to-day referral development. The Directory page has a live “Zone” filter and an “ Local competitors” quick-filter chip, so this stays usable going forward instead of being a one-time snapshot.



## Part 2 — Competitive Landscape, County by County

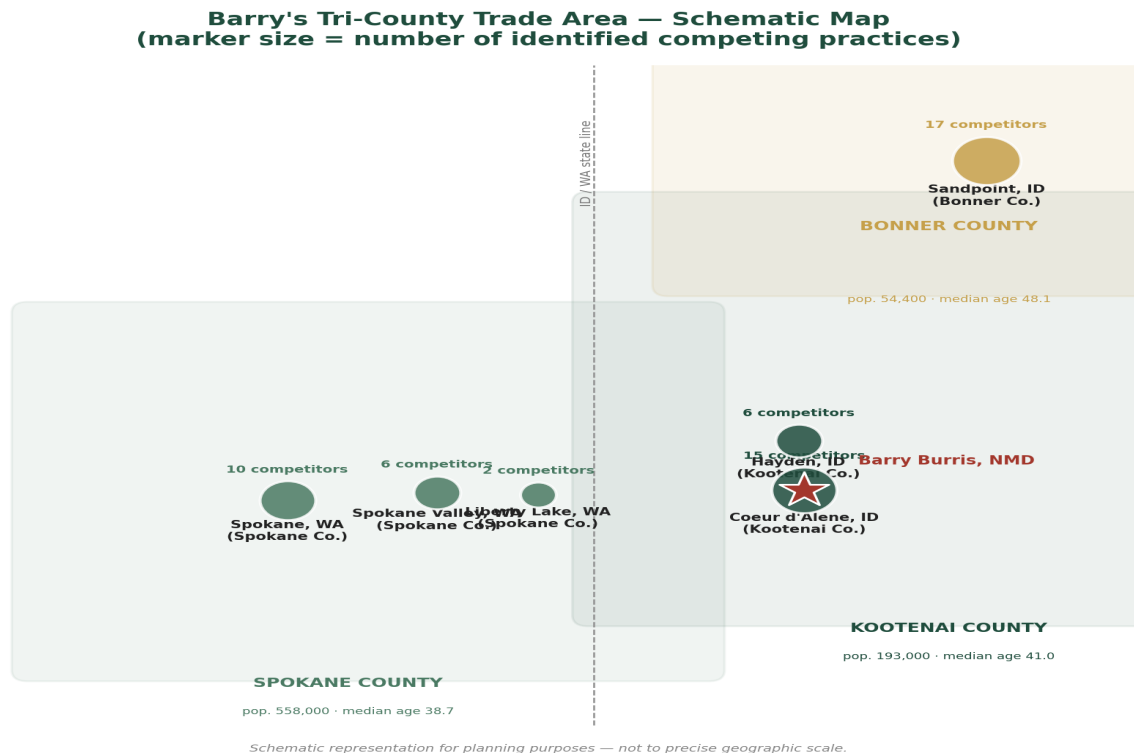


Figure 1. Barry's tri-county trade area (schematic — marker size = competitors identified per city).

### 2.1 Kootenai County — Home Market

Kootenai is Barry's home turf, and it's crowded. Between naturopaths, hormone/peptide clinics, and med-spas, **21 practices** were identified competing for the same patient inside the county — more than in Bonner or Spokane individually, despite Kootenai having a fraction of Spokane's population.

**Naturopathic / functional-medicine (6):** Lakeside Holistic Health, Restore Wellness, CDA Acupuncture & Holistic Healing, Coeur d'Alene Healing Arts, Vital Health (40+ years — the longest local tenure found anywhere in the tri-county area), Overland Wellness (concierge model).

**Hormone / peptide / concierge medicine (9):** Gameday Men's Health, Northwest Integrated Health, Hormone Logics, USHGH Peptides, Klinik, Tri Valley Health & Aesthetics, Reignite Health NW, Coeur Vitality, North Idaho Direct Primary Care.

**Med-spa / IV / weight loss (6):** North Idaho Med Spa (strongest visible review presence locally), Renew Aesthetics/RBar Functional Wellness, NorthLife Aesthetics & Wellness (opened 2022, fastest-growing entrant), Beauty At The Lake, Signature Aesthetics, Prime IV Hydration.

**Demographics:** population ~193,000 (2025), growing ~1.8%/year, up 28% over the last decade. Median age 41.0, with 20.6% of the population 65+. Median household income \$81,861. Uninsured rate 9.4%.

**What's driving demand:** Kootenai's growth is substantially retiree in-migration — the 65+ population more than doubled between 2001 and 2019, over half of it from people moving in, not aging in place. The dominant origin states are California and Washington. Housing costs run 14–19% above the national average, filtering for real



purchasing power. The county also has a well-documented, decades-long pattern of politically conservative, self-pay-oriented in-migration — relevant because that population skews toward wanting autonomy from institutional/insurance-based healthcare, which is Barry's exact positioning.

## 2.2 Bonner County — Sandpoint, Secondary Market

Bonner is smaller (54,400 people) but structurally different in a way that matters: it's the **oldest** of the three counties (median age 48.1, with roughly 42% of the population 55 or older) and it has a huge second-home/seasonal-resident base — 86% of vacant housing units are for seasonal or recreational use, and average home values (\$657K per Zillow) run well above Kootenai's.

**17 competing practices** were identified, clustered almost entirely in downtown Sandpoint. Three are true naturopathic peers (Green Mountain Medicine, Restorative Naturopathy — est. 2008, and Aspen Naturopathic Health, active status unconfirmed). The rest split between concierge/DPC-style practices with a functional lean (Sandpoint Regenerative Medicine, Lakeshore Health, Sandpoint Premier DPC, Cornerstone DPC, Integrative Health Solutions) and a dense med-spa/aesthetics cluster (Signature Aesthetics — 108+ reviews, the strongest-reviewed practice in the county, Glow Aesthetics, Meadowsweet Medical, 7B-IV Trinity Wellness, Lakeside Medicine, Applegate Health Care).

**Migration signal:** Census migration-flow data (2015–2019) already showed a net gain of 270 movers from LA County alone. Real-estate brokers quoted in a January 2025 *Bonner County Daily Bee* piece independently described continued relocation from California and western Washington as an ongoing pattern, not a one-time COVID spike.

**Health profile:** Bonner ranks higher-middle tier for health outcomes among Idaho's 44 counties. Regional BRFSS data shows 29.8% adult obesity, 8.0% diabetes, and 19.9% depression prevalence — with a 2022 community health survey naming mental health as the single most-cited community concern (58% of respondents), ahead of obesity or access to care.

Bonner County commutes into Kootenai in meaningful numbers (1,300+ jobs held across the county line each direction), functionally linking the two as one regional labor and patient market.



## 2.3 Spokane County — Adjacent Major Metro

Spokane is the market Kootenai and Bonner both feed into and compete against — six times Kootenai's population (558,000) at a younger median age (38.7). It's also the most institutionally competitive of the three: healthcare is a top-three employment anchor (Providence and MultiCare together employ over 10,000 locally), and **18 competing practices** were identified.

Four confirmed local NMD peers, plus a larger tier of MD/DO/NP-led hormone, peptide, and longevity practices (9 identified) — including The Metabolic Institute (est. 2005, the longest-tenured functional-medicine practice found anywhere in this research, explicitly marketing to “the Spokane/Coeur d'Alene area”) and Moore Family Clinic's dedicated “Health and Longevity Center” (opened 2024). Five med-spa/IV/aesthetics operators round out the list, plus two national franchise brands (Prime IV, Aligned Modern Health) with a local footprint.

**The finding that should change how Barry thinks about Spokane specifically:** Clinic 5C — a Spokane-based cosmetic-plus-functional-medicine practice with a strong review record (RealSelf 4.8★/393 reviews) — has apparently opened or expanded a Coeur d'Alene location as of mid-2026. That converts Clinic 5C from an adjacent-market competitor into a direct, in-market one; worth confirming directly (a call or site visit), since this surfaced from search results rather than a verified announcement.

**Health/spending signals:** diabetes 9.2%, obesity 34.0%, depression 26.9% — a larger, somewhat less healthy population than either Idaho county, with genuine unmet demand. Cost of living runs about 2% below the national average overall, but healthcare costs specifically run 2% above average, suggesting willingness to pay more for care is already somewhat established locally.

## 2.4 Combined Tri-County View

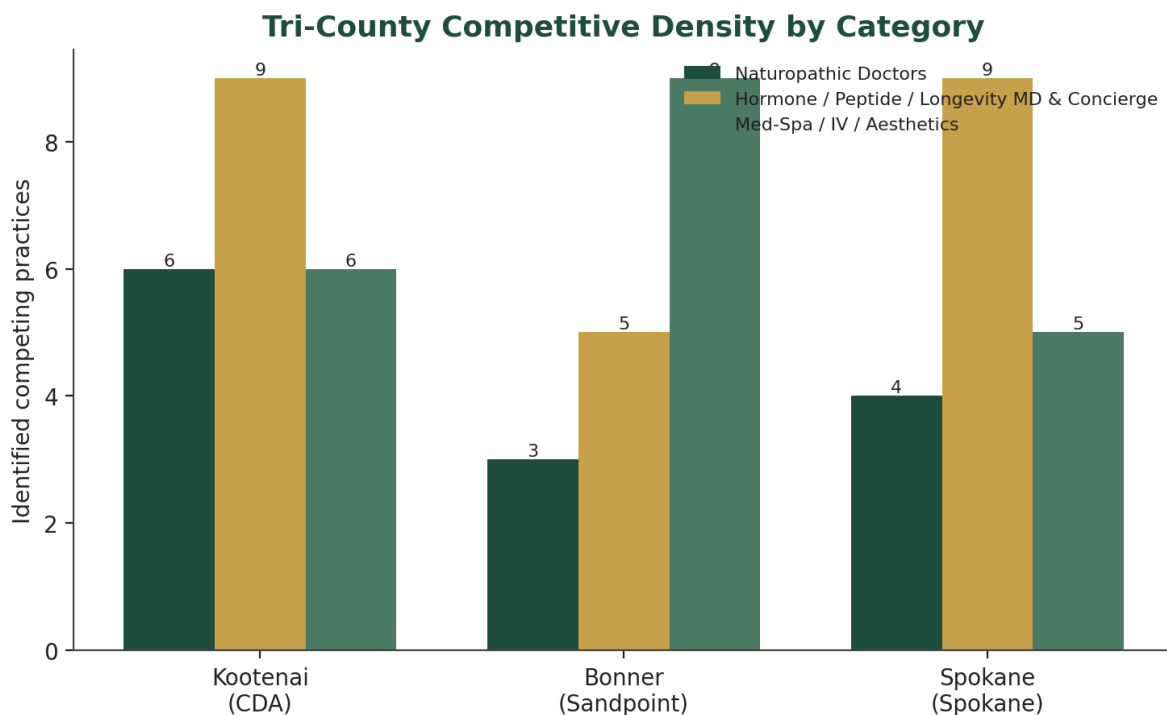


Figure 2. Competitor density by category and county.



### Share of 56 Tri-County Competitors by County

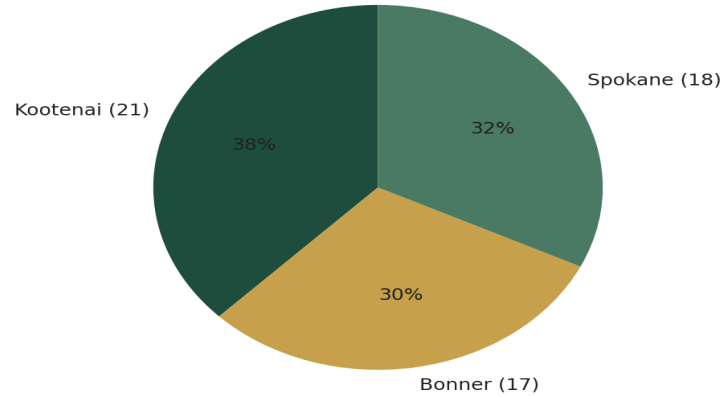


Figure 3. Share of the 56 tri-county competitors, by county.

Across all three counties, **56 practices compete with Barry** for some slice of the same patient, once the definition of “competitor” is widened past naturopaths-only to include the hormone/peptide/longevity-MD and med-spa/IV categories functionally offering the same thing under a different license. Kootenai accounts for the largest single share (21, 38%), followed closely by Spokane (18, 32%) and Bonner (17, 30%) — meaning Bonner, despite being by far the smallest county by population, punches roughly at parity with the other two on competitor count. That's a market-maturity signal: Sandpoint's wellness-spending culture is disproportionate to its population, likely driven by its older, more affluent, seasonal-resident base.

The tri-county area totals roughly **806,000 people**, with the 65+ population alone exceeding **150,000** — a population base larger than many mid-size metro areas, concentrated within an hour's drive of Barry's Coeur d'Alene practice.



## Tri-County Demographic Snapshot

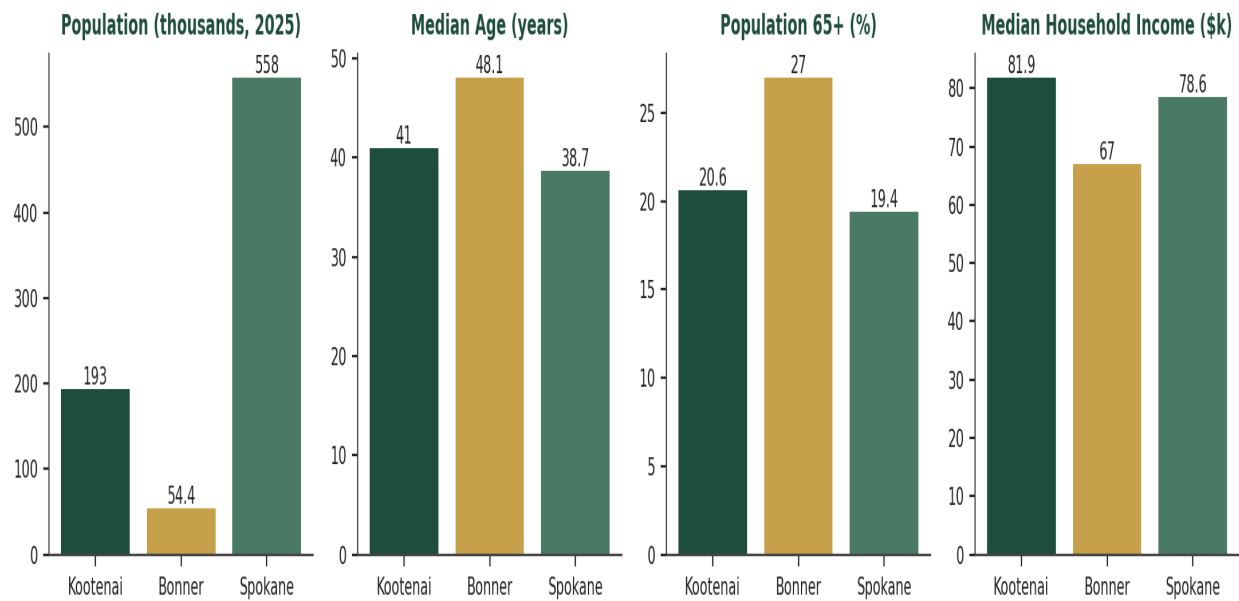


Figure 4. Tri-county demographic snapshot.

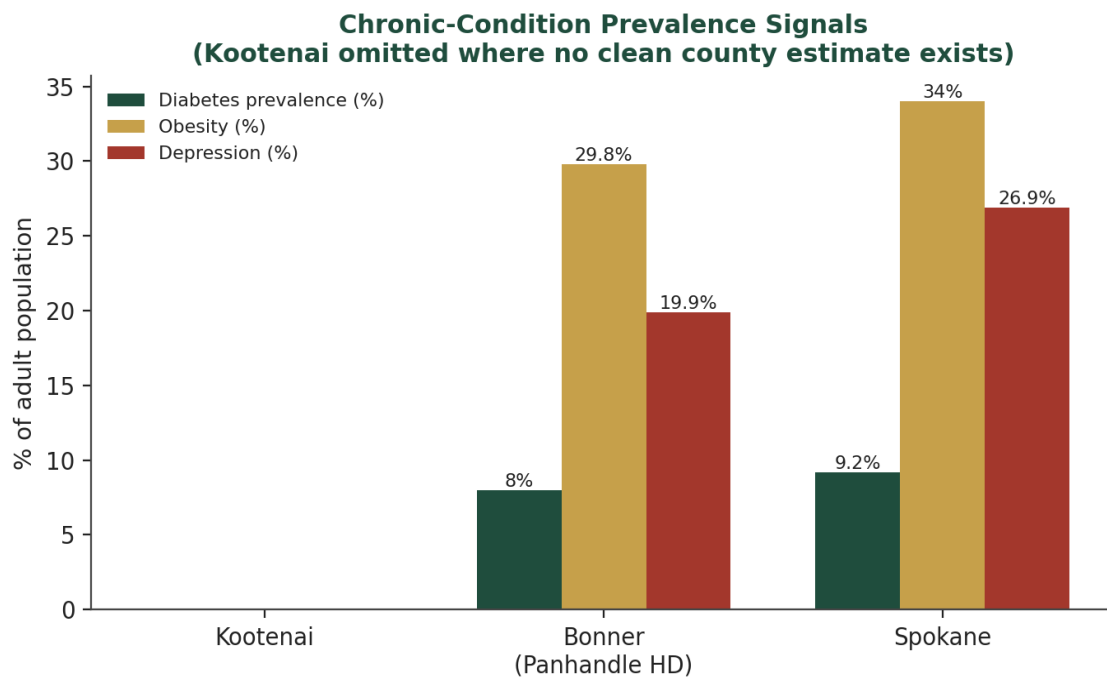


Figure 5. Chronic-condition prevalence signals (Kootenai omitted where no clean county estimate exists — flagged, not fabricated).





## Part 3 — Target Partner Profile (Referral ICP)

Not every complementary practitioner on Barry's list is worth the same outreach effort. Based on what actually differentiates the practices that would send Barry patients (rather than compete with him for them), the ideal referral partner profile looks like this:

| Fit Criterion                                         | Why It Matters                                                                                                                                                                                                              |
|-------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1. Non-competing modality                             | Chiropractic, acupuncture/Chinese medicine, physical therapy, and mental-health/counseling practices are structurally non-competing — they don't offer hormone optimization, peptide therapy, or naturopathic primary care. |
| 2. Independent ownership                              | Not health-system-affiliated. Independent practices control their own referral relationships; a provider inside a Providence- or MultiCare-affiliated group has far less latitude to refer out.                             |
| 3. Cash-pay or hybrid billing                         | Practices whose patients are already comfortable paying out of pocket (or HSA/FSA) are referring into a population that won't balk at Barry's own cash-pay model.                                                           |
| 4. Established 3+ years, active digital presence      | A practice with real reviews and an updated site is one that's still operating and reachable — this quietly excludes several "thin"-flagged entries until verified.                                                         |
| 5. Already serving an older or wellness-oriented base | A physical therapist specializing in mobility/longevity, or a counselor leaning toward high-achieving adults managing stress, is already talking to Barry's target patient before Barry ever meets them.                    |
| 6. Geographic overlap                                 | Kootenai, Bonner, or the Spokane Valley/Liberty Lake corridor closest to the Idaho line — a referral relationship works best when the referring practice's patients can realistically make the drive.                       |

**Highest-priority partner types, in order:** (1) chiropractors and physical therapists with an active-adult/mobility focus, (2) acupuncturists and Chinese-medicine practitioners already positioned around chronic pain or fertility (natural overlap with Barry's own hormone/fertility work), (3) mental-health counselors and therapists serving professional/executive clients, (4) concierge and direct-primary-care practices that explicitly don't do hormone or nutrition work themselves and need a specialist to refer to for it, (5) med-spas whose menu is purely aesthetic (injectables, laser) with no hormone or IV-therapy overlap — these are functionally allies, not competitors, despite superficially adjacent branding.

**Who to deprioritize:** any practice on the "thin" list until verified, any practice inside a hospital system's employed-physician network, and — obviously — every entry now flagged *competitor: true* in the directory.



## Part 4 — Probable Patient / Customer Market: Segmentation

This is a separate exercise from the referral-partner work above — it's about who actually walks in the door, broken out by geography, demographics, and psychographics (the “why,” not just the “who”). Four segments cover the bulk of the realistic tri-county patient base.

### Segment A — The Relocated Retiree Optimizer

| Dimension      | Detail                                                                                                                                                                                                                                                                       |
|----------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Geography      | Kootenai lakefront/golf communities (Hayden Lake, Coeur d'Alene proper), Bonner County second-home and lakefront areas.                                                                                                                                                      |
| Demographics   | 60–75, retired or semi-retired, household net worth well above local median, relocated from California or western Washington within the last 5–10 years.                                                                                                                     |
| Psychographics | Values autonomy and self-directed healthcare; skeptical of institutional/insurance-based medicine; motivated by “staying active and independent,” not disease treatment; responsive to longevity/vitality framing; found via word of mouth and local “best of” publications. |
| Why it matters | Largest, most self-pay-ready segment — the 65+ population across all three counties exceeds 150,000 and is growing faster than the general population in two of the three counties.                                                                                          |

### Segment B — The Midlife Executive / Entrepreneur

| Dimension      | Detail                                                                                                                                                                                                                               |
|----------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Geography      | Coeur d'Alene, Hayden, Spokane Valley, Liberty Lake — anywhere with a remote-work or small-business-owner concentration.                                                                                                             |
| Demographics   | 40–55, working professional or business owner, household income well above county median, time-constrained.                                                                                                                          |
| Psychographics | Hormone/energy/weight concerns framed as performance issues; wants concierge-level access over a waiting-room experience; research-driven — arrives having self-diagnosed via labs or wearables; price-sensitive on time, not money. |
| Why it matters | Most likely found via targeted digital content (the “Briefs from AE” positioning) and peer referral within local business networks.                                                                                                  |

### Segment C — The Chronic Complexity Patient

| Dimension      | Detail                                                                                                                                                                                           |
|----------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Geography      | Distributed across all three counties — this segment travels for the right provider.                                                                                                             |
| Demographics   | Any age, most commonly 35–65; already been through conventional care without resolution (autoimmune symptoms, unexplained fatigue, gut issues, hormone imbalance not caught by standard panels). |
| Psychographics | Highly research-driven, often frustrated with prior care; will pay out of pocket for a provider who looks at root cause.                                                                         |
| Why it matters | Least demographically predictable, highest-intent — most likely to arrive via referral from a chiropractor, acupuncturist, or counselor (directly ties to the Part 3 referral strategy).         |



## Segment D — The Second-Home / Seasonal Wellness Visitor

| Dimension      | Detail                                                                                                                                                                                                             |
|----------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Geography      | Bonner County specifically (Sandpoint, Schweitzer Mountain corridor, Lake Pend Oreille shoreline).                                                                                                                 |
| Demographics   | Owns a second home or seasonal property; typically affluent, 45–70; splits time between Bonner County and Puget Sound or California.                                                                               |
| Psychographics | Wants convenient, low-friction access during time in the area — telehealth and flexible/house-call scheduling appeal directly; values privacy; less price-sensitive given the second-home ownership signal itself. |
| Why it matters | Smaller in absolute numbers but disproportionately valuable per patient — none of the 17 Bonner County competitors identified market to seasonal residents as a distinct segment.                                  |



## Part 5 — What This Means, Practically

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1. **Referral outreach should start with the 71 local-zone contacts, not the full 868** — and within that, prioritize the Part 3 profile over raw list order. The extended-network 797 are a real asset (conference contacts, expansion-market intelligence, patient-relocation referrals) but not this quarter's priority.
2. **The 17 local NMD competitors are not outreach targets** — they're the honest baseline for how crowded this specific credential is locally. Barry's differentiation has to come from something other than "I'm also a naturopathic doctor" — the 56-practice combined competitive set makes clear that hormone/peptide/longevity positioning is already contested by MDs, NPs, and med-spas who don't carry the ND credential at all.
3. **Bonner County is underrated as a market.** It's the smallest by population but has the oldest, most affluent, most seasonally-flexible patient base of the three — and Segment D (seasonal wellness visitors) is currently unclaimed by any competitor identified there.
4. **Watch Clinic 5C directly.** If they have genuinely opened a Coeur d'Alene location, they are now Barry's single most credible in-market competitor — physician-led, strong reviews, and already straddling both the Spokane and Kootenai markets the way Barry himself might want to.
5. **The "Briefs from AE" hub section** (built alongside this report) gives Barry a place to keep this kind of intelligence live and update it over time, rather than treating it as a one-time PDF that goes stale.

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*Prepared by Accelerated Experiences, LLC. Sources cited throughout: U.S. Census Bureau (QuickFacts, ACS, Decennial Census), Idaho Department of Labor, Washington State Office of Financial Management, County Health Rankings & Roadmaps, CDC PLACES, Kootenai Health Community Health Needs Assessment (Dec. 2025), Bonner General Health Community Health Needs Assessment (2022), Zillow, Redfin, individual practice websites and public review platforms. Figures without a clean sourced point estimate are noted as such rather than approximated.*